

CARZEPIN TABLET 200MG

VICAR13-var (MY)

Description

Round, 10.0 mm in diameter, white uncoated tablet, bevel-edged, flat faces, "HOVID" embossed on one face and scored on the other.

Composition

Carbamazepine 200 mg/tablet

Pharmacodynamics

In animal studies, carbamazepine raises the threshold for electrically and chemically induced convulsions. After-discharge activity is reduced, especially in the amygdaloid nucleus and hippocampus, and the cortico-thalamic threshold is raised. The motor cortex is relatively unaffected. Decreased post-tetanic potentiation has been demonstrated in the rabbit spinal cord, and has decreased synaptic transmission in the spinal trigeminal nuclei in cats. Decreased appreciation of induced pain has been shown in guinea pigs and rabbits. Carbamazepine will convert ECG patterns to normal in digoxin-induced tachycardia by prolongation of A-V Conduction and reduction of Purkinje cell automaticity. In human studies, suppression of spike foci has not been demonstrated. Anticonvulsant and anti-neuralgic effects are believed to result primarily from depression of synaptic transmission.

Pharmacokinetics

Absorption

Carbamazepine is almost entirely absorbed from the gastro-intestinal tract, the major proportion of the administered dose being recoverable in the urine. The absorption from carbamazepine tablets is slow but virtually complete.

Blood Concentration

After an oral dose of 6mg/kg, peak serum concentrations are about 4ug/ml at 7 hours and 6.5ug/ml at 3 hours. During therapy with about 12.5mg/kg daily for 6 months, mean plasma concentrations for carbamazepine and its 10, 11 -epoxide metabolite are about 5 and 1ug/ml respectively.

Therapeutic concentrations are in the range 3 to 6ug/ml.

Side-effects are suffered by 50% of subjects with blood concentrations in the range of 8.5 to 10ug/ml.

Half-Life

Plasma half-life is 21 to 53 hours. In infants the half-life is reduced to 8 to 27 hours. After chronic treatment the plasma half-life is significantly reduced.

Distribution

Carbamazepine enters the cerebrospinal fluid, crosses the placenta, and is secreted in milk. In blood it is taken up by red blood cells.

Protein-binding - 70 to 80% bound to plasma protein and the epoxide metabolite is about 50% bound.

Metabolic Reactions

Hydroxylation to form 10, 11 -dihydro-10, 11 -dihydroxy-5H-dibenz [b, f] azepine-5-carboxamide. Epoxidation to form the 10, 11 -epoxide, and glucuronic acid conjugation. In man, carbamazepine may increase the metabolism of warfarin and phenytoin.

Excretion

Less than 1% of a dose is excreted unchanged in the urine; about 25% is excreted in the urine as the 10, 11 -dihydroxy metabolite.

Indications

For treatment of:

- Partial seizures with complex symptomatology (psychomotor, temporal lobe)
- Generalized tonic-clonic seizures (grand mal)
- Mixed seizure patterns
- Pain due to trigeminal neuralgia and glossopharyngeal neuralgia.

Contraindications

- Not recommended in patients with atrioventricular (AV) heart block, blood disorders, and history of bone marrow depression.
- Use in nursing mothers should generally be avoided.

Warning And Precautions

- Caution in geriatric patients and in patients with coronary artery disease, diabetes mellitus, glaucoma, history of hematologic adverse reactions to other medications, urinary retention, cardiac, hepatic or renal disease.
- Abrupt discontinuation in a responsive epileptic patient may precipitate convulsions or status epilepticus; gradual

withdrawal is recommended.

- Plasma carbamazepine concentration determinations is recommended periodically as a guide to efficacy and safety.
- Carbamazepine should be given with caution to patients who are hypersensitive to tricyclic antidepressants.
- This medication may cause drowsiness. Caution when driving or operating machinery. Avoid alcoholic beverages.
- Safety for use in pregnancy has not been established.
- Severe dermatological reactions including Stevens-Johnson syndrome and toxic epidermal necrolysis (Lyells syndrome) have been reported with carbamazepine. Patients treated with carbamazepine should be closely monitored for signs of hypersensitivity reactions, particularly during the first month of therapy. Immediate discontinuation of therapy should be made when a cutaneous reaction occur. Potential for an increase in risk of suicidal thoughts or behaviours.

Pregnancy and Lactation

Not recommended in pregnancy and lactation.

Interaction with Other Medicaments and Other Forms of Interaction

- Monoamine oxidase (MAO) inhibitors should be discontinued at least 14 days before carbamazepine therapy is initiated.
- Dosage adjustments may be necessary when used concurrently with anticoagulants, coumarin or indandione derivative, doxycycline, or tricyclic antidepressants.
- Effects of oral contraceptives, barbiturates, benzodiazepines, hydantoins, primidone, succinimides or valproic acid may be decreased during concurrent therapy with carbamazepine.
- Co-administration of erythromycin, propoxyphene or troleandomycin may increase carbamazepine plasma levels and toxicity.

Main Side / Adverse Effects

Dizziness, drowsiness, ataxia, nausea or vomiting.

Overdose

Clinical features:

Dizziness, ataxia, nystagmus, abdominal discomfort, urinary retention, hyperreflexia.

Treatment:

Treat overdosage by emesis or gastric lavage, if appropriate; with the necessary supportive measures, if required.

Dosage And Administration

Adults:

Anticonvulsant

Initial: Oral, 200 mg two times a day on the first day, the dosage then being increased as needed.

Maintenance: Oral, 800 mg to 1.2 gram a day, in 3 or 4 divided doses.

Analgesic

Initial: Oral, 100 mg two times a day on the first day, the dosage then being increased as needed.

Maintenance: Oral, 200 mg to 1.2 grams a day, in 3 or 4 divided doses.

Children:

Anticonvulsant

Below 6 years

Initial: Oral, 5 mg per kg body weight per day, the dosage being increased as needed.

Maintenance: Oral, 10 to 20 mg per kg body weight per day, in 3 or 4 divided doses.

6 to 12 years

Initial: Oral, 100 mg two times a day on the first day, the dosage being increased as needed.

Maintenance: Oral, 400-800 mg a day in 3 or 4 divided doses, not exceeding 1 gram daily.

The information given here is limited. For further information, consult your doctor or pharmacist.

Storage : Store below 30°C. Protect from moisture.

Presentation / Packing:

Blister pack of 100's; Bottle of 100's, 500's, 1000's (Not all presentations is available locally)

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